

Community Acquired Pneumonia — Quick Reference

Two questions drive everything: **how severe** (site of care) and **MDR risk factors** (how broad to cover). Adapted from TeachIM (CC BY-NC 4.0); reflects 2019 IDSA/ATS + 2025 ATS update.

Microbiology

- **GPCs:** *S. pneumoniae* (#1), *S. aureus*.
- **GNRs:** *H. influenzae*, *M. catarrhalis*.
- **Atypicals:** *Legionella*, *Chlamydia*, *Mycoplasma*.
- **Viruses:** influenza, COVID-19 (major cause).

MDR risk (MRSA/PsA): prior MRSA/PsA isolation, or hospitalization + IV abx within 90 d. **HCAP retired (2019).**

Severity & Site of Care

Tool	Decides	Cut-offs
PSI (pref.)	Site	I–II outpt; III obs; IV–V inpt
CURB-65	Site	0–1 outpt; 2 admit; 3–5 inpt
ATS	ICU	>1 major or ≥3 minor

CURB-65: Confusion, Urea>20, RR≥30, BP<90, age≥65.

ATS major: septic shock on pressors, mech vent. **Minor (≥3):** RR>30, P:F<250, multilobar, confusion, BUN>20, WBC<4k, plt<100k, T<36, hypotension → fluids.

Inpatient Management

Scenario	Cx?	Coverage
Severe	Yes*	Standard + MRSA + PsA
Prior MRSA/PsA	Yes	Standard + MRSA ± PsA
Hosp+IV abx <90d	Yes	Standard CAP
No RF	No	Standard CAP

*blood, sputum, *Legionella* + pneumococcal urine Ag, MRSA nares.

- **Standard:** IV BL (amp-sulbactam/ceftriaxone) + azithro/doxy, or resp FQ.
- **MRSA:** vancomycin/linezolid. **PsA:** zosyn/cefepime/ceftazidime/aztreonam/meropenem (keep atypical cov).
- De-escalate at 48 h if cx / MRSA nares negative.

Outpatient Management

Group	Regimen
No comorbid/RF	Amoxicillin 1 g TID or doxycycline
Comorbidities	Augmentin/cephalosporin + azithro/doxy, or resp FQ

Comorbidities: chronic heart/lung/liver/renal dz, DM, alcoholism, malignancy, asplenia.

No macrolide monotherapy (US pneumococcal resistance).

Adjuncts & Duration

- **Procalcitonin:** NOT for starting abx (sens ~38–91%); may guide early stop.
- **Steroids (2025 ATS):** give for *severe* CAP (hydrocort <400 mg/d ×5–7 d, CAPE COD); **not** non-severe; dexamethasone if COVID.
- **Duration:** 5–7 d, ≥5 d AND clinically stable. Longer if extrapulm, PsA/Mycobacteria/Burkholderia, or necrotizing/abscess/empyema.

Do / Avoid

- **DO** use PSI/CURB-65 for site, ATS criteria for ICU.
- **DO** draw cultures in severe CAP / MDR risk; narrow promptly.
- **AVOID** empiric MRSA/PsA without a risk factor.
- **AVOID** using procalcitonin to withhold antibiotics.
- **AVOID** azithromycin monotherapy and low-dose amoxicillin.